

## Decision

Excessive data collection by the Cyprus Sports Organization during the issuance of a Health Card for the sport of chess

I refer to the complaint submitted to my Office on November 13, 2022, by XXX (hereinafter referred to as the "Complainant") against the Cyprus Sports Organization (hereinafter referred to as the "CSO") and to all relevant correspondence regarding the above matter, which forms part of the case files. Based on the duties and powers conferred upon me by Articles 57 and 58 of Regulation (EU) 2016/679 on the protection of natural persons with regard to the processing of personal data and on the free movement of such data (hereinafter referred to as the "Regulation"), I note the following:

### A. Facts of the case

2. The complaint stated, among other things, the following:

- i. The CSO requires the provision of detailed sensitive personal health data for the purpose of issuing a Health Card for the intellectual sport of chess.
- ii. Specifically, for the issuance of the Health Card, a medical examination by a cardiologist, medical history, hereditary/family medical history, chest X-ray, electrocardiogram, and general blood tests are required. The Complainant considers this collection excessive, inappropriate, and a clear violation of the provisions of the Regulation, particularly the principle of data minimization.
- iii. The issuance of the Health Card is a necessary prerequisite for a chess player to participate in events organized by the Cyprus Chess Federation and local chess clubs. As claimed by the Complainant, the CSO does not rely on any scientific basis for this requirement, nor on any European or international practice.

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- iv. The Complainant perceives this specific requirement as coercion to provide personal data.
- v. The Complainant approached the Data Protection Officer of the CSO requesting that it be examined whether the provisions/decisions regarding the issuance of Health Cards comply with the principles governing the processing of personal data as defined in the Regulation, but the response received was unsatisfactory.

3. Along with the complaint form, the Complainant sent to my Office the correspondence exchanged with the CSO's Data Protection Officer, the forms "Medical Examination for Health Card" and "Return Form 'Medical Examination for Health Card'".

4. For the investigation of the complaint, my Office sent a relevant letter to the CSO on December 9, 2022, through which it notified the submission of the complaint against it and requested its positions regarding the claims made by the Complainant. On January 9, 2023, the initial positions of the CSO were received, which stated, among other things, the following:

- i. The CSO Health Card and the pre-sport check constitute a safety measure for athletes of all ages, of any sport. The purpose of issuing the Athlete Health Card is -
  - to prevent Sudden Cardiac Death in athletes during physical exercise,
  - to ensure early diagnosis and prevention of pathological conditions that do not exclude full or limited athletic activity but require special precautions and monitoring.
- ii. For the determination of the medical and paraclinical examinations and the generally required medical procedure, a 5-member Scientific Committee of the Supreme Council of Sports Health (hereinafter referred to as the "Committee") has been established, under the powers of the Board of Directors of the CSO (Article 5(e) of Law 41/1969).
- iii. "The CSO Athlete Health Card (which is issued in a plastic card format) is mandatory and constitutes a necessary prerequisite for the participation of athletes of any age, level, category, and sport in training and competitions of recognized Sports Clubs and Sports Federations. The Sports Federations have the absolute responsibility for verifying the validity of the Health Card of the athletes and therefore it must be presented to the secretary and/or the relevant referee(s) and/or observer(s) of

any kind of competition and sporting event, in accordance with the

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regulations and the competition announcements of the recognized Sports Federations”

iv.

“The issuance of Health Cards is governed by the Sports Organization Law of 1969 (41/69 Part Three, points (f) and (k)), as well as the regulations, guidelines, procedures, and circulars issued by the Health Cards Department, following decisions of the Board of Directors of the Sports Organization. Therefore, the collection, processing, and storage of data (medical and non-medical) of all applicants for the issuance of Health Cards constitute compliance of the Organization as referred to in Articles 6.1(c) and Article 9.2(h) of the General Regulation on Personal Data – GDPR.

v.

The Board of Directors of the Organization, exercising the powers granted to it by the Law, specifically Article 5.2 (of the “PART THREE PURPOSE OF THE ORGANIZATION AND POWERS OF THE BOARD OF DIRECTORS”), has established the Scientific Committee of the Supreme Council of Athlete Health (SCASH), which is the competent body for determining the medical and paraclinical examinations, and the generally required medical procedure for the submission of an application and evaluation for the issuance of Health Cards for Athletes of the Sports Organization. The SCASH Committee, following meetings and consultations with the National Sports Organization (NSO), in its session dated 08/04/2021, which was validated by the Board of Directors of the Organization (23/06/2021), decided to amend the medical and paraclinical examinations of athletes of the NSO, as well as other intellectual sports who have reached the age of 34 (...).”

vi.

“Personal data concerning the application and evaluation for the issuance of a Health Card (including Parental Consents, Authorizations, and Receipt Forms) are retained for up to six (6) months from the expiration of the issued Health Card, provided that there are no obligations or claims from the involved parties and/or provided that there is no ongoing audit or judicial process related to the case. Furthermore, the deletion of personal data occurs automatically upon the approval and issuance of a new Health Card or in the event that your application is recorded as pending for a period exceeding six (6) months from the date of submission.

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The above applies also in cases where six (6) months have passed from the date of issuance and submission:

- Of the form “Medical Examination for the issuance of a Health Card,” the medical and paraclinical examinations as well as the necessary supporting documents for the issuance of a Health Card.
- Of the supplementary/referral examinations required by the Medical Committee.
- Of all certificates and documents required by the Department for further evaluation and handling of the application.

vii.

“(...) the Health Card for Athletes of the Sports Organization and the pre-sport check constitute a safety measure for the participation of athletes of all ages, who regularly engage, whether amateur or professional, in any kind of sport.”

5. Attached to the above response was a letter from the President of the Scientific Committee of SCASH dated December 28, 2022, a table of required medical examinations and documents for the issuance of a Health Card, and a copy of the Sports Organization Law of 1969 (41/1969).

6. According to the SCASH Committee, whose positions have been analyzed in its letter dated December 28, 2022:

i.

The Sports Organization is the Supreme Sports Authority of the Republic of Cyprus, responsible for the issuance of Health Cards for Athletes.

ii.

The mission of the Sports Organization through the issuance of Health Cards is the prevention of

Sudden Cardiac Death in athletes during physical exercise, as well as the timely diagnosis and prevention of pathological conditions that do not exclude full or limited athletic activity but require special precautions and monitoring.

iii.

The goal of the Health Cards is to subject all athletes regardless of age or sport, who systematically or amateurishly engage in sports, to a predetermined medical check aimed at detecting health problems that may endanger their lives not only during exercise but in any activity.

iv.

For the determination of the necessary medical and paraclinical examinations of athletes, the SCASH Scientific Committee has taken into account the relevant literature, scientific and research databases, all scientific publications and recommendations, and the updated guidelines of global and European standards concerning pre-sport and pathological medical checks.

v.

The SCASH Committee, after meetings with the federations of intellectual sports, including the Chess Federation in the presence of its cardiologist, and following a study, has agreed and decided on the prerequisite examinations when submitting the application for the issuance of Health Cards for athletes participating in intellectual sports.

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vi. The questions and examinations to which the athlete is subjected for the issuance of the Health Card aim at the timely identification and exclusion of serious causes that potentially endanger life during systematic exercise and/or activity.

vii. The examinations to achieve the above goals have been determined for mental sports by a decision of the Scientific Committee of ASYA, following meetings with the Chess Federation, in the presence of its cardiologist.

viii. "Each country shapes its relevant legislation in consultation with the competent sports authority regarding pre-sport checks for their systematic participation in competitions and activities. Cyprus is considered one of the pioneers in the field of sudden death prevention (...) with many years of experience in pre-sport checks."

ix. The matter has also been discussed with the Health Committee of the Parliament, where the importance of pre-sport checks and the prevention of sudden death, as well as the issuance of the Health Card for its prevention, was analyzed and documented by the Cyprus Sports Organization (KOA) and the ASYA Committee, regardless of the type of activity (physical or mental).

x. The ASYA Committee concludes in the letter dated December 28, 2022, that "In no case is there an issue of personal data. The process falls under the practices of medical practice. The recording of pathological findings forms a database for research purposes for the benefit of reliable statistics and the formation of an incident registry, adhering to the rules of the bioethics committee. The extraction of pathological data will contribute to the improvement of practices followed in the prevention of SCD (Sudden Cardiac Death) as well as maximizing the conditions for ensuring the lives of citizens/athletes."

7. Following the receipt of the above positions, my Office contacted KOA again on February 3, 2023, expressing some initial views on the matter and requesting further clarifications regarding KOA's claim that the medical examinations required by KOA for the issuance of Health Cards are in accordance with European/international standards and/or guidelines and are not more than those required to satisfy the purpose of issuance. On February 17, 2023, the ASYA Committee stated the following:

i. According to international literature, pre-sport checks aim to prevent sudden death during exercise and diagnose pathological conditions that do not permit athletic participation. This is a cardiological check performed before the commencement of any athletic activity for professional or recreational reasons. To exclude even the slightest possibility of such an incident, a pre-sport cardiological check is requested before participation in athletic activities.

ii. Prevention of sudden cardiac death through the implementation of pre-sport cardiovascular checks is recommended by the European and American Cardiology Societies.

iii. The guidelines of the European Cardiology Society consider the performance of an

electrocardiogram and its careful interpretation, along with taking a medical history and physical examination, essential in pre-sport checks.

iv. The detection of heart diseases in athletes requires the application of a specific screening protocol, in which individual and family history, clinical examination, and the electrocardiogram play a prominent role.

v. For the determination of the medical procedure, the following were studied:

a. The "Guide for the Evaluation of Electrocardiograms in Young People & Athletes" and the literature referenced therein,

b. The guidelines of the European Cardiology Society and the American Cardiology Society,

c. The Decision of the Hellenic Republic dated 3/8/2018 on the subject "Athlete Health Card,"

d. The findings of the 42nd Panhellenic Cardiology Congress,

e. Other bibliographic sources (e.g., Eur Heart J 2005, European Journal of Preventive Cardiology 2017).

vi. In all mental sports, for athletes over the age of 34, the submission of a medical certificate (Medical Examination Form for the issuance of Health Card), ECG with interpretation, blood tests, etc., is sufficient for the purpose of issuing a Health Card, which was agreed upon with the Chess Federation, in the presence of its cardiologist, unlike other sports for which additional examinations are required. Attached to the above response was the Decision of the Hellenic Republic dated 3/8/2018 on the subject "Athlete Health Card," the program of the 42nd Panhellenic Cardiology Congress, and a copy of the Cyprus Sports Organization Law of 1969 (41/1969).

8. On March 15, 2023, the Complainant was informed of the above and of some of my initial positions, and on March 17 and May 12, 2023, stated, among other things, the following:

i. "However, it seems that you have relied entirely on information and opinions provided to you by the Cyprus Sports Organization and the Cyprus Chess Federation, based on the existing legal framework, and you have not conducted independent research, such as seeking the agreement of an independent medical expert or the opinion of individuals and...

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Organizations that are not directly involved. Any independent parties may, of course, agree with the ASYA, but it does not appear that you sought confirmation of the ASYA's decisions and took them for granted and above any doubt. (...) Although Cypriot law grants the ASYA the relevant powers, I believe that this control was reasonable to conduct given that for chess - unlike other physical sports - it is not common practice in other EU countries to require such data and examinations.

ii. The Decision of the Hellenic Republic dated 3/8/2018 has been amended. With a subsequent sports law, Law 4726/2020, Article 12, paragraph 9, chess players are exempted from the obligation to certify their health and issue a health card as a prerequisite for their participation in training and competitions. Verbally, this sports law states: "Athletes of competitive bridge and draughts are exempt from the obligation to certify their health and issue the health card of the third paragraph of this law, as a prerequisite for their participation in training and competitions." (Draughts is an older Greek word for chess).

iii. It is noted that in the recent announcement of the nationwide individual championships for young men and young women (publication date 3/3/2023 - <https://www.chessfed.gr/archives/11744>), it confirms the relevant regulation stating verbatim: "For participation, a valid, renewed – activated Sports Identity Card of the E.S.O. is required. It is clarified that the Sports Identity Card is considered renewed – activated if the corresponding renewal – activation fee (10 €) or the registration fee paid for the issuance of a new Card (new registration – issuance from 1/1/2023 and onwards) has been paid. It is emphasized that according to Article 12 of Law 4726 (Government Gazette 181 A' 18/9/2020), chess players are exempt from the obligation to certify their health and issue a health card as a prerequisite for their participation in training and competitions."

iv. The Hellenic Chess Federation collects far fewer data for athlete registration compared to what the Cyprus Sports Organization (KOA) requires from Cypriot chess players through the issuance of the Health Card.

v. The Complainant also raised the question, "Any medical examinations are always useful since they may indicate some pathology and prevent issues, but when can their cost be justified for the purposes they serve?" As he states, the cost of medical examinations is borne by the athletes and their families, not by the KOA or the General Health System (GESY) - since if the state were to assume this cost, it might lead to a more in-depth and detailed examination of the issue, and the conclusions might be different.

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vi. Due to the relative lack of specific literature and studies concerning preventive medical checks in intellectual sports, the Complainant posed the following questions on April 23, 2023, to the online artificial intelligence application ChatGPT: "Is pre-participation screening necessary for chess?" and "Is pre-participation screening necessary for American football?" From the responses he received, which he included in his letter, he concluded that: "In summary, despite not being an expert, the ASYA's view on the medical examinations required for chess and intellectual sports does not seem to be generally adopted, not only in the competitive practice of chess abroad but also by many cardiologists and research tools such as ChatGPT."

vii. It appears that the non-implementation of the Health Card by some federation belonging to the KOA may have serious consequences, such as the drastic reduction of funds allocated to a federation or even withdrawal from the KOA. For the chess federation, a potential refusal might also lead to financial strangulation, inability to fund national missions abroad, etc. Thus, the Federation likely had to consider other factors in its decision to comply with the measure and not just the usefulness of the specific checks for the sport.

viii. In addition to the above, the issuance of a Health Card requires military enlistment documents and military discharge even for intellectual sports. The KOA's website related to the Health Card <https://cyprussports.org/health-journal/> contains several forms linking the Health Card to the fulfillment of military obligations - in case a male is of an age that he should enlist, in contrast to the simple form that needs to be filled out for the issuance of an athlete card from the Hellenic Chess Federation.

ix. "Objectively, I think it is difficult to justify this specific requirement of the Organization for the submission of military enlistment documents in relation to intellectual sports - beyond the fact that this is done within the framework of a motivation (or threat) for someone to fulfill their military obligations. Of course, I do not dispute that this is done within the legal framework that has been approved. However, it is worth questioning where the right of the Organization and the state to request information unrelated to the purpose for which it is needed stops. That is, if in the future the KOA requests a clean criminal record (in an attempt, for example, to exclude former criminals from sports to reduce violence) or proof that the athlete has fully met their tax obligations (in an attempt to combat tax evasion), will this also be compatible with the regulation that your office was appointed to protect?"

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x.

"(...) I want to point out that there is an asymmetry between the application of personal data protection rules for the public sector and for the private sector. In the public sector, most actions and powers derive from the Parliament and the Executive authority, in the form of legislation. One way to respond to my complaint is that the specific practice of the CCA is 'Correct because it is law' and 'The law is law because it is correct.' In other words, it is a circular argument. That is, even practices that may need to be reconsidered and revised and that may be contrary to certain broader principles, because they are covered by the veil of legality, are not subject to challenge. Conversely, in the private sector, shareholders, general assemblies, boards of directors, and executive directors decide how to conduct their operations autonomously and do not need their decisions to be approved by the Parliament and the Executive authority in the form of new laws. Thus, since they are not covered by the veil of legislation, it is easier to challenge them and for private organizations to be held accountable for their actions and omissions."

9. Taking into account the above positions of the Complainant, my Office addressed a relevant inquiry to the supervisory authorities of Portugal, Malta, Greece, Ireland, and Italy regarding the procedures/rules for pre-sport checks in chess (whether the issuance of a health certificate is required and what data is collected), in order to further investigate the issue. The Data Protection Supervisory Authorities of Greece, Ireland, and Malta confirmed that no medical data is requested or medical checks are conducted for the issuance of any medical certificate, so that chess players can participate in official competitions. The Supervisory Authority of Italy stated that specific legislative provisions exist only for "masters professionals," noting, of course, that the FSI - Italian Chess Federation is a member of CONI (Italian Olympic Committee), therefore it implements and applies all its guidelines, including codes of conduct on anti-doping issues. Finally, the Supervisory Authority of Portugal also noted the Anti-doping Regulation, under which athletes may be subject to periodic checks, emphasizing that the principle of proportionality should govern any issuance of a health certificate, meaning that the certificate should be proportional to the sport in which it will be used/for which it is issued.

10. After receiving the above positions, on February 28, 2024, I issued the Preliminary Decision, in which I found a prima facie violation of Article 5(1)(c) of the Regulation by the CCA, and I determined that it should be reassessed whether the issuance of a Health Card for athletes in chess is necessary and if so, to document whether the collected sensitive personal data is relevant and minimal, adhering to the principle of proportionality. On April 10, 2024, I received the positions of the CCA regarding the aforementioned Decision.

11. According to the CCA:

"1. The Board of Directors of the Organization, exercising the powers granted to it by the relevant legislation (Article 5.2. (f), (g), and (k) of Law (41/69), has established the Scientific Committee of the Supreme Council for Athlete Health (SCSA) and the Medical Committees for Athlete Evaluation (MCAE).

2. It is noted that every athlete of a recognized sports federation or association who trains and competes systematically, either amateur or professional, is required to possess a Health Card. The purpose of the Health Card and the examinations conducted for its issuance is the prevention of Sudden Cardiac Death in athletes. At the same time, it also achieves the timely diagnosis and prevention of pathological conditions that do not exclude full or limited athletic activity but require special precautions and monitoring.

3. The SCSA, according to its terms of reference, is responsible among other things for "[...] determining the medical and paraclinical examinations and the overall required medical procedure for the submission of an application for the issuance of a CCA Athlete Health Card."

4. Furthermore, following recommendations from the SCSA, the Board of Directors of the Organization has established subcommittees, the Medical Committees for Evaluation (MCAE), which meet and review and evaluate secondarily the medical and paraclinical examinations submitted by athletes wishing to obtain a Health Card and approve the issuance of the Health Card as well as its duration of validity.

5. According to the regulations and procedures established by the Organization following the recommendations of the SCSA, all athletes wishing to obtain a CCA Health Card must be examined primarily by a physician of their choice and undergo the necessary pre-sport medical and cardiological check-up.

6. The Cyprus Chess Federation (CCF), as a recognized Sports Federation, according to the provisions of the Cyprus Sports Organization Law of 1969 (Law 41/69) as subsequently amended, as well as the interpretations contained in Article 2 of the legislation (competition, sport, etc.), has the obligation to comply with and implement all regulations and decisions issued by the CCA, including the issuance of the CCA Health Card.

Sports and Sudden Cardiac Death

7. Sports and exercise, beyond their undeniable benefits, can also be a source of health problems primarily from cardiovascular issues.

system with a leading tragic event being sudden cardiac death (SCD). Specifically, the death of an athlete represents a serious blow to the overall social consciousness, as athletes personify the epitome of vigor and health.

8. Sudden cardiac death is the most common cause of non-traumatic death among athletes. In a large study by Corrado and his colleagues<sup>1</sup>, it has been estimated that participation in competitive sports increases the risk of sudden cardiac death by 2.5 times compared to individuals who do not engage in sports.

9. Epidemiological data have shown that the causes of SCD vary by age. For athletes under 35 years of age, the most common causes of SCD are Hereditary Cardiovascular Diseases (HCD), with the main representatives being Cardiomyopathies, Electrical Arrhythmia Syndromes (Channelopathies), and anomalous coronary artery origin. With the exception of anomalous coronary origin, these diseases are usually due to some mutation in the genome and are inherited from parents to their offspring. The peculiarity of these diseases is that they often do not present symptoms, with the first and last manifestation being SCD. The Health Questionnaire, with targeted questions from the hereditary and personal history of the athlete and with the help of the Electrocardiogram (ECG), aims to detect the possible existence of an HCD in an athlete and thus protect them from the risk of a cardiovascular event during exercise.

10. For athletes over 35 years of age, studies have shown that the main cause of sudden cardiac death is coronary artery disease, although some of the aforementioned clinical entities<sup>3</sup> cannot be excluded. This is also the reason why a Stress Test is required at these ages to detect asymptomatic coronary artery disease that may potentially cause an acute coronary event during exercise at these ages.

Purpose - Issuance of Health Questionnaires by the Cyprus Sports Organization (CSO)

11. One of the most important parameters supporting sports in our country has been the adoption of the Health Questionnaire by the sports community of Cyprus (Federations and Clubs).

12. The issuance of the Health Questionnaire by a State body and/or its adoption by Federations and Clubs constitutes a groundbreaking action in sports not only in Europe but also in the global sports arena.

13. Over the past 24 years, more than 150,000 Health Questionnaires have been issued, while today more than 15,000 Health Questionnaires are issued annually due to the increase in the population of athletes.

14. At this moment, the Health Questionnaire represents the safest safeguard regarding the health of those engaged in sports (both physically and mentally). It is issued under the guidance and control of a permanent Committee, which is appointed by the Board of Directors of the Organization and which delineates and modernizes the future policies of the Health Questionnaire based on international literature and recommendations from Scientific Societies.

15. Based on the above, the purpose of the specific questions for the issuance of the Health Questionnaire is to extract the minimum basic information that would raise suspicion of the existence of an HCD and to inform the athlete about the recommended precautions or avoidance of specific athletic activities. To make the above clearer, some examples are mentioned:

(a) In the event that the family history indicates SCD in a young family member, there is a high likelihood that an HCD may be present that has been transmitted to other family members, and given that these diseases do not present symptoms, this individual may be at risk.

(b) The same applies in cases of a coronary event at an age < 50 years in a family member.

(c) The finding of a patent foramen ovale in a young athlete prohibits athletic activities that involve diving. The targeted information from the medical history in the CSO Health Questionnaire aims precisely to assist the physician in identifying underlying serious asymptomatic cardiovascular disease and consequently preventing pathological conditions that do not necessarily exclude full or limited athletic activity but require special precautions and monitoring.

15.1. As mentioned in the introduction, the issuance of a Health Questionnaire is mandatory for athletes of recognized sports federations and clubs who train and compete systematically, whether amateur or professional.

15.2. Furthermore, the recommendations of the CSO / ASYA to the general public who engage in

sports and exercise systematically outside of federated and club sports are to undergo the necessary pre-sport medical and cardiological examination and submit an application for the issuance of a Health Questionnaire.

15.3. The pre-sport medical and cardiological examination and the issuance of the Health Questionnaire, as we mentioned in previous letters, have been established based on International and European standards as well as the guidelines of the European and American Cardiology Societies<sup>4</sup>.

15.4. Individuals participating in competitive-level activities, whether these involve sports that require physical activity or sports that do not require physical activity, expose their bodies to conditions of maximum or nearly maximum exertion.

15.5. The designation of an exercise as "intense" does not equate to maximum effort. The level of intense exertion, whether physical, "mental," or psychosomatic, is defined by the attending physician according to the characteristics of the individual being examined (see individual, hereditary history, physical examination, etc., which are required during the primary medical examination).

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15.6. The above is particularly emphasized as "intense" exercise has been associated with an increased rate of sudden death, especially in individuals over the age of 35, primarily due to coronary artery disease.

Issuance of Health Cards and KYSO

16. As we have mentioned in our previous letters, for the medical and cardiological examinations required by KYSO athletes, the K.O.A. and the A.S.Y.A. have had repeated meetings with KYSO, in the presence of its Cardiologist. A consultation was thus held with the Board of Directors of KYSO in the presence of their scientific collaborator, who fully agreed and consented to the procedure followed in the case of KYSO athletes.

17. Therefore, following discussion, study, and mutual acceptance, it was decided that for athletes over the age of 34 participating in KYSO training and competitions, the requirements for the issuance of Health Cards would be minimized and "restricted" concerning sports that require physical exercise.

18. Additionally, it was jointly decided that the duration of the issuance of the Health Card for KYSO athletes would be increased from the two (2) years applicable to sports involving physical exercise to three (3) years.

19. As a result of the above, the requirements for the issuance of Health Cards for KYSO athletes over the age of 35 have been limited to what is absolutely necessary for the issuance of a Health Card and are proportionate to achieving the purpose for which they are required, namely for the prevention of Sudden Cardiac Death (see paragraph 11).

19.1. For athletes up to 34 years of age, the same medical examinations are required as for all sports, as we will explain further, chess has been categorized in studies and research as having the same biochemical, physical, and other factors as all sports.

20. Furthermore, due to the three-year validity of the Health Card for KYSO athletes, these athletes undergo the required medical examinations every three (3) years.

21. For the evaluation of an application for the issuance of a Health Card for KYSO athletes over the age of 34, the following are required:

- K.O.A. Form "Application for Medical Examination for Health Card."
- Electrocardiogram (ECG) with a report from a Cardiologist.
- General Blood Tests
- Biochemical Analyses (only the 1st time for athletes over 34 years old)
- Chest X-ray (only the 1st time for athletes over 34 years old)
- Issuance for three (3) years.

21.1. In contrast, for sports involving physical exercise, the issuance of a Health Card for federal and club athletes over 34 years old requires the following, proportionate to achieving the purpose for which they are required, namely for the prevention of Sudden Cardiac Death:

- K.O.A. Form
- ECG with a report
- Exercise Stress Test



- General Blood Tests
- Biochemical Blood Analyses (the 1st time over 34 years old)
- Chest X-ray (the 1st time over 34 years old) Issuance for up to two (2) years.

Pre-competitive Medical Cardiological Check-up KYSO & Issuance of Health Card

22. The effect of Mind Sports on the cardiovascular system during a match, particularly in Chess, has not been studied in depth. Based on data from MEDLINE PUB MED, where all valid medical publications are recorded, among 36 million publications, there are only two references regarding the effect of Chess on the cardiovascular system. These studies (refs. 1 & 2) emphasize the impact of the sport in a demanding chess match on cardiovascular variability - Heart Rate Variability (HRV) - Changes in HRV can cause arrhythmias, particularly atrial fibrillation.

23. Based on publications in the Journal of Sports Medicine & Doping Studies (ref. 3), we have highlighted in yellow the findings that the biochemical, physiological, psychological aspects, and changes in the central nervous system in individuals competing in chess are equivalent to all other sports. The study makes specific references to metabolic load, changes in heart rate (HRV), and many other physiological parameters.

23.1. Publications and other bibliographic sources for studies and research on Chess athletes (refs. 4 & 5), which we have also highlighted in yellow.

24. As mentioned above, the level of "intensity" and the load of individuals during exercise is determined by the attending physician, depending on the characteristics of the individual being examined. Even among athletes of the same sport or sports team, the same intensity does not have the same load on the cardiovascular system of individuals but depends on the physiology of each person individually.

25. Based on the above, the sport of chess has equivalent biochemical, physiological, and other requirements as all "classical" sports, including football, and appropriate precautions must be taken for the health of the chess player in a mentally demanding competition.

26. Decision of the Scientific Committee of A.S.Y.A. dated 27.3.2024

The Scientific Committee of A.S.Y.A., after reviewing your letter dated 28.2.2024 and its content, the attached bibliographies and publications, as well as all the literature and international standards concerning pre-athletic medical and cardiological examinations, at its meeting on 27.3.2024, decided to further minimize the pre-requirements for the issuance of Health Cards for KYSO athletes in relation to other sports federations as follows:

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## MEDICAL EXAMINATIONS AND CERTIFICATES

### KYSO

(athletes for all ages)

Sports Federation of sports that require physical activity

Athletes up to 11 years old

Athletes aged 11-34 years

Athletes aged 34 years and older

1.

Medical Examination Form

ASYA KOA "Application for Issuance of Health Card"

(fully completed and signed by the examining Physician and the applicant)

V

V

V

V

2.

Electrocardiogram at rest (signature and stamp of physician)

V

V

V

V

3.

Stress Test

.....

.....

.....

V

4.

Echocardiogram

In case a heart murmur is diagnosed

In case a heart murmur is diagnosed

In case a heart murmur is diagnosed

In case a heart murmur is diagnosed

5.

Full Blood Count

V (for athletes over 11 years old)

V

V

6.

Biochemical Blood Analyses

(only upon first application)

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.....

V

V

7.

Blood Type (only upon first application)

V

V

V

V

8.

Chest X-ray and report

(only upon first application)

.....

.....

V

DURATION OF VALIDITY OF HEALTH CARD

Up to 3 years

Up to 3 years

Up to 3 years

Up to 2 years

It is understood that:

1. The decision of ASYA pertains only to the prerequisite medical and paraclinical examinations related to the pre-sport medical and cardiological assessment of KYSO athletes for the issuance of a Health Card and not to other elements and data related to the legislation and regulations for the issuance of a Health Card.

2. For KYSO athletes over 34 years old, the KOA Form and the Electrocardiogram at rest will be completed by a Cardiologist.

3. The validity period of the Health Card for KYSO athletes will be for 3 years, except in cases where pathological findings are identified during their secondary evaluation by the Medical Evaluation

Committees, for which the athlete must provide supplementary medical and paraclinical examinations according to the Medical Committee's report.

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4. According to the above decision, KYSO athletes are required to submit an application for the issuance of a Health Card every three (3) years, along with the submission of the "Medical Examination for Issuance of Health Card" Form, Electrocardiogram with the report of the examining Physician, and General Blood Analyses.

#### Conclusions

27. The questions and examinations required for the issuance of a Health Card are proportional to the purposes they serve, namely the detection of cardiovascular problems that may endanger an athlete's life and the prevention of SCD (Sudden Cardiac Death). These risks are independent of the type of sport - mental or physical.

28. The obligation to issue a Health Card applies only to athletes participating in competitions and training of recognized sports federations and clubs.

29. From 01/01/2019 to 31/12/2023, KOA/ASYA evaluated 62,511 applications for the issuance of Health Cards, and currently, 48,112 Health Cards are in effect.

30. Both the European Society of Cardiology and the American College of Cardiology agree that it is imperative to screen athletes for both medical and ethical/legal reasons.

31. This position is supported by the actions and deeds of both the EU ASYA and its members, as well as all members of the Board of Directors of the Organization over time.

#### Bibliography:

1. Corrado, D., Basso, C., Rizzoli, G et al. Does sports activity enhance the risk of sudden death in adolescents and young adults? J Am Coll Cardiol 2003; 42: 1959-63
2. Maron BJ, Doerer JJ, Haas TS, Tierney DM, Mueller FO. Sudden deaths in young competitive athletes: analysis of 1866 deaths in the United States, 1980-2006. Circulation. 2009;119:1085-1092
3. Maron BJ, Epstein SE, Roberts WC. Causes of sudden death in competitive athletes. J Am Coll Cardiol. 1986;7:204-214
4. Maron, BJ., Thompson, P.D., Ackerman, MJ. et al. Recommendations and considerations related to preparticipation screening for cardiovascular abnormalities in competitive athletes: 2007 update: a scientific statement from the American Heart Association Council on Nutrition, Physical Activity, and Metabolism: endorsed by the American College of Cardiology Foundation. Circulation 2007; 115:1643-55.
5. Corrado, D., Pelliccia, A., Bjornstad, H.H. et al. Cardiovascular pre-participation screening of young competitive athletes for prevention of sudden death: proposal for a common European protocol. Eur Heart J 2005; 26: 516-24

Attached to the above letter are the scientific articles with the following titles:

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1. The stress of chess players as a model to study the effects of psychological stimuli on physiological responses: an example of substrate oxidation and heart rate variability in man
2. Differences Between High vs. Low Performance Chess Players in Heart Rate variability During Chess Problems
3. Biochemistry and Psychology of Chess and Classical Physical Exercise: Concurring or Conflicting Evidence?
4. Arrhythmia risk assessment using heart rate variability parameters in patients with frequent ventricular ectopic beats without structural heart disease.
5. Effects of Official Chess Competition on Salivary Cortisol and Mood Swings in Adolescent Girls: A Win-Loss Approach

#### B. Legal Framework:

12.1. According to Article 4 of the Regulation,

"1) 'personal data': any information relating to an identified or identifiable natural person ('data

subject'); an identifiable natural person is one who can be identified, directly or indirectly, in particular by reference to an identifier such as a name, an identification number, location data, an online identifier, or to one or more factors specific to the physical, physiological, genetic, psychological, economic, cultural or social identity of that natural person,

2) 'processing': any operation or set of operations which is performed on personal data or on sets of personal data, whether or not by automated means, such as collection, recording, organization, structuring, storage, adaptation or alteration, retrieval, consultation, use, disclosure by transmission, dissemination or otherwise making available, alignment or combination, restriction, erasure or destruction,

(...)

7) 'controller': the natural or legal person, public authority, agency or other body which, alone or jointly with others, determines the purposes and means of the processing of personal data; where the purposes and means of such processing are determined by Union law or the law of a Member State, the controller or the specific criteria for its designation may be provided for by Union law or the law of a Member State

(...)

15) 'data concerning health': personal data which relate to the physical or mental health of a natural person, including the provision of health care services, and which reveal information about the health status of that person"

12.2. According to Article 5 of the Regulation,

"Principles relating to the processing of personal data

1. Personal data shall be:

- a) processed lawfully, fairly and in a transparent manner in relation to the data subject ('lawfulness, fairness and transparency'),
- b) collected for specified, legitimate purposes and not further processed in a manner that is incompatible with those purposes; further processing for archiving purposes in the public interest or for scientific or historical research purposes or statistical purposes shall not be considered incompatible with the initial purposes in accordance with Article 89(1) ('purpose limitation'),
- c) adequate, relevant and limited to what is necessary in relation to the purposes for which they are processed ('data minimization'),
- d) accurate and, where necessary, kept up to date; every reasonable step must be taken to ensure that personal data that are inaccurate, having regard to the purposes for which they are processed, are erased or rectified without delay ('accuracy')
- e) kept in a form which permits identification of data subjects for no longer than is necessary for the purposes for which the personal data are processed; personal data may be stored for longer periods, provided that the personal data will be processed solely for archiving purposes in the public interest, for scientific or historical research purposes or for statistical purposes, in accordance with Article 89(1) and provided that appropriate technical and organizational measures are applied as required by this Regulation to safeguard the rights and freedoms of the data subject ('storage limitation'),
- f) processed in a manner that ensures appropriate security of the personal data, including protection against unauthorized or unlawful processing and against accidental loss, destruction or damage, using appropriate technical or organizational measures ('integrity and confidentiality').

2. The controller shall be responsible for, and be able to demonstrate compliance with, paragraph 1 ('accountability')."

12.3. According to Article 6 of the Regulation:

1. Processing shall be lawful only if and to the extent that at least one of the following applies:

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- a) the data subject has consented to the processing of their personal data for one or more specific purposes,
- b) the processing is necessary for the performance of a contract to which the data subject is a party or to take steps at the request of the data subject prior to entering into a contract,

- c) the processing is necessary for compliance with a legal obligation to which the data controller is subject,
- d) the processing is necessary to protect the vital interests of the data subject or another natural person,
- e) the processing is necessary for the performance of a task carried out in the public interest or in the exercise of official authority vested in the data controller,
- f) the processing is necessary for the purposes of the legitimate interests pursued by the data controller or a third party, except where such interests are overridden by the interests or fundamental rights and freedoms of the data subject which require protection of personal data, especially if the data subject is a child.

The provision f) of the first paragraph does not apply to processing carried out by public authorities in the performance of their tasks.

#### 12.4. According to Article 9 of the Regulation:

##### “Processing of special categories of personal data

1. The processing of personal data revealing racial or ethnic origin, political opinions, religious or philosophical beliefs, or trade union membership, as well as the processing of genetic data, biometric data for the purpose of uniquely identifying a natural person, data concerning health, or data concerning a natural person's sex life or sexual orientation is prohibited.

2. Paragraph 1 does not apply in the following cases:

- a) the data subject has given explicit consent to the processing of those personal data for one or more specific purposes, unless Union or Member State law provides that the prohibition referred to in paragraph 1 may not be lifted by the data subject,
- b) the processing is necessary for compliance with legal obligations and the exercise of specific rights of the data controller or the data subject in the field of employment law and social security and social protection law, provided that it is permitted by Union or Member State law or by collective agreement under national law providing appropriate safeguards for the fundamental rights and interests of the data subject,
- c) the processing is necessary to protect the vital interests of the data subject or another natural person, if the data subject is physically or legally incapable of giving consent,
- d) the processing is carried out, with appropriate safeguards, in the course of the legitimate activities of a foundation, association, or other not-for-profit body with a political, philosophical, religious, or trade union aim and on the condition that the processing relates solely to the members or former members of the body or to persons who have regular contact with it in connection with its purposes and that the personal data are not disclosed outside that body without the consent of the data subjects,
- e) the processing concerns personal data which have been manifestly made public by the data subject,
- f) the processing is necessary for the establishment, exercise, or defense of legal claims or whenever courts are acting in their judicial capacity,
- g) the processing is necessary for reasons of substantial public interest, based on Union or Member State law, which is proportionate to the aim pursued, respects the essence of the right to data protection, and provides for appropriate and specific measures to safeguard the fundamental rights and interests of the data subject,
- h) the processing is necessary for purposes of preventive or occupational medicine, for the assessment of the working capacity of the employee, medical diagnosis, the provision of health or social care or treatment, or the management of health or social care systems and services based on Union law or the law of a Member State or pursuant to a contract with a health professional and subject to the conditions and safeguards referred to in paragraph 3,
- i) the processing is necessary for reasons of public interest in the area of public health, such as protecting against serious cross-border threats to health or ensuring high standards of quality and safety of health care and of medicinal products or medical devices, based on Union law or the law of a Member State, which provides for appropriate and specific measures to protect the rights and freedoms of the data subject, in particular the professional secrecy, or

j) the processing is necessary for archiving purposes in the public interest, for scientific or historical research purposes, or for statistical purposes in accordance with Article 89 paragraph 1 based on Union or Member State law, which are proportionate to the aim pursued, respect the essence of the right to data protection, and provide for appropriate and specific measures for the

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**\*\*Ensuring the fundamental rights and interests of the data subject.\*\***

3. The personal data referred to in paragraph 1 may be processed for the purposes provided for in paragraph 2 point (h), when such data are processed by or under the responsibility of a professional who is subject to the obligation of professional secrecy under Union law or the law of a Member State, or under rules established by competent national authorities, or by another person who is also under an obligation to maintain confidentiality under Union law or the law of a Member State, or under rules established by competent national authorities.

4. Member States may maintain or establish further conditions, including restrictions, regarding the processing of genetic data, biometric data, or data concerning health.

12.5. According to Article 58 of the Regulation, “2. Each supervisory authority shall have all of the following corrective powers:

- a) to issue warnings to the controller or the processor that intended processing operations are likely to infringe provisions of this Regulation,
- b) to issue reprimands to the controller or the processor when processing operations have infringed provisions of this Regulation,
- c) to order the controller or the processor to comply with the requests of the data subject to exercise their rights under this Regulation,
- d) to order the controller or the processor to bring processing operations into compliance with the provisions of this Regulation, if necessary, in a specific manner and within a specified time limit,
- e) to order the controller to notify the personal data breach to the data subject,
- f) to impose a temporary or definitive limitation, including a ban on processing,
- g) to order the rectification or erasure of personal data or restriction of processing pursuant to Articles 16, 17, and 18 and to order the notification of such actions to recipients to whom the personal data have been disclosed pursuant to Article 17(2) and Article 19,
- h) to withdraw a certification or to order the certification body to withdraw a certificate issued in accordance with Articles 42 and 43 or to order the certification body not to issue certification, where the certification requirements are not met or are no longer met,
- i) to impose an administrative fine pursuant to Article 83, in addition to or instead of the measures referred to in this paragraph, depending on the circumstances of each individual case,
- j) to order the suspension of data flows to a recipient in a third country or to an international organization.”

12.6. According to the Cyprus Sports Organization Law of 1969 (41/1969):

“5.-(1) The purpose of the Organization is to promote extracurricular physical education and sports in general on the island, to cultivate and develop the Olympic ideal and the spirit of sportsmanship among the people, to promote the island's sports internationally within the framework of existing relations with foreign authorities and sports organizations, and in cooperation with these authorities and organizations, to improve its position in the international sports movement, as well as to promote an action plan for healthy sports.

(2) To achieve the purpose of the Organization, the Board of Directors, in accordance with the provisions of this Law, has the authority to:

(...)

(σ) establish any committees, bodies, or organs whose establishment may be deemed desirable.

(ζ) assign to any committee, body, or organ established by it the exercise of any of the powers granted to it by this Law.

(...)

(k) determine the issuance of health cards for athletes and provide for the general procedure for the

medical examination of each athlete member, sports federation, or sports club:

Provided that the health card shall in no case be issued if the athlete is a conscript who has been called up and has enlisted in the National Guard, in accordance with the provisions of the National Guard Law, from the year 2019 onwards and holds, under this Law, a temporary discharge, either due to unfitness for military service (I/5) or due to a deferral granted to him for the fulfillment of the obligation of military service, due to a disease/condition related to his mental health:

Provided further that, in the event that a health card has been issued to a conscript athlete who has been called up and has enlisted in the National Guard, in accordance with the provisions of the National Guard Law, from the year 2019 onwards, and subsequently he was granted a temporary discharge for the reasons mentioned in the above reservation, the act of issuing the health card shall be revoked by the issuing authority, either from the date of commencement of the...

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By virtue of the Cyprus Sports Organization (Amendment) (No. 3) Law of 2021 or from the date of any subsequent temporary dismissal of the conscript:

It is further understood that the act of revocation, which is referred to in the second reservation, is immediately communicated both to the athlete conscript and to the relevant sports federation and sports club to which he belongs.

### C. Reasoning

13. According to Article 5(1) of the Cyprus Sports Organization Law of 1969 (41/1969), the purpose of the Organization is "the promotion of extracurricular physical education and sports in general on the island, the cultivation and development of the Olympic ideal and the spirit of sportsmanship among the people, the athletic promotion of the island abroad within the framework of existing relations with foreign authorities and sports organizations, and in cooperation with these authorities and organizations, the improvement of this position in the international sports movement, as well as the promotion of an action plan for healthy sports."

14. For the achievement of the above purpose, pursuant to Article 5(2) of Law 41/1969, the Board of Directors of the Cyprus Sports Organization has the authority, among others, to determine what is required for the issuance of Health Cards for athletes and the procedure for the medical examinations that must be followed.

15. Based on the same article, the Board of Directors of the Organization, exercising its powers, may establish a committee, body, or instrument to which it may delegate the exercise of any of its responsibilities. Based on these provisions, the Board of Directors of the Cyprus Sports Organization decided to establish the Scientific Committee "Supreme Council for Athlete Health (SCASH)."

16. According to the publicly available information on the website of the Cyprus Sports Organization, "The terms of reference and the responsibilities of the SCASH Committee have been defined by a decision of the Board of Directors of the Organization dated November 3, 2011. The following are briefly mentioned:

1. The SCASH is the competent body that addresses issues of preventive examination of athletes with the ultimate goal of preventing sudden death and is responsible for the issuance of health cards under the name Health Cards SCASH.

2. The SCASH is the competent committee of the Organization for determining the required procedure for the medical examination for the issuance of the health card.

3. It is responsible for monitoring developments and practices followed in the science of Athlete Health globally and particularly in the area of sudden cardiac death and prevention of athlete health and recommends modifications to the health card issuance procedures where deemed necessary."

17. Taking into account the above information, pursuant to Article 4 of the Regulation, the collection, storage, and any other use of medical data or other personal data of athletes during the issuance of Health Cards constitutes a processing act. The Complainant, like all athletes, as the individuals whose data is being processed, constitutes the data subjects, and correspondingly the Cyprus Sports Organization, as the organization that determines the purpose and means of processing, is the data

controller.

18. Despite the legality of the issuance of Health Cards, as it is based on Article 5 of Law 41/1969, any processing act carried out within the framework of this issuance must be governed by the Principles of data processing as defined in Article 5 of the Regulation. Among these, the principle of purpose limitation and data minimization (Article 5(1)(b) and (c) of the Regulation). Pursuant to these, the medical data collected for the issuance of Health Cards must be collected for specified, explicit, and legitimate purposes, must not be further processed in a manner incompatible with those purposes, must be limited to what is necessary, and must be adequate and relevant to the purpose. I emphasize that the determination of general purposes does not automatically legitimize excessive data collection.

19. During the investigation of the complaint, to evaluate the above principle, I requested from the Cyprus Sports Organization, as the data controller, to inform me about the purpose of issuing the Health Cards. According to the final positions of the SCASH Committee submitted before me, "The purpose of the Health Card and the examinations conducted for its issuance is the prevention of Sudden Cardiac Death in athletes. At the same time, it also achieves the timely diagnosis and prevention of pathological conditions that do not exclude full or limited athletic activity but require special precautions and monitoring."

20. The necessity of preventing Sudden Cardiac Death in athletes was justified based on a specific study (Corrado and his collaborators), according to which it has been estimated that participation in competitive sports increases the risk of sudden cardiac death by 2.5 times compared to individuals who do not engage in sports.

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21. In its initial statements, the Cyprus Sports Organization (KOA) indicated that the collection of medical data for the issuance of the Health Card aims to detect health problems that may endanger the lives of athletes, not necessarily during exercise, but generally during any activity, a purpose which I had deemed quite broad and general in my prima facie Decision. Considering my positions, as outlined above, the purpose of issuing the Health Cards was limited to the prevention of Sudden Cardiac Death in athletes.

22. As I had pointed out in my prima facie Decision, since the issuance of Health Cards is a necessary prerequisite for athletes' participation in events organized by their recognized Federations and aims to protect the health of athletes, it should be proportionate to the sport. Taking into account the positions of the 5 other supervisory authorities I consulted, I concluded in my Decision that the necessity of issuing the Health Card for chess players should be reassessed and the proportionality of the collected data should be duly documented.

23.1. Considering my above indications, the ASYA Committee, with specific references to studies, noted that -

- the impact of the sport in a demanding chess competition on cardiovascular variability has been emphasized, changes which may cause arrhythmias, particularly atrial fibrillation,
- the biochemical, physiological, psychological aspects, and changes in the central nervous system in individuals competing in chess are equivalent to all other sports,
- individuals participating in competitive-level activities, whether they involve sports requiring physical activity or those that do not, expose their bodies to conditions of maximum or nearly maximum load,
- the level of "intensity" and load of individuals during exercise is determined by the attending physician, depending on the characteristics of the individual being examined. Even among athletes of the same sport or sports team, the same intensity does not impose the same load on the cardiovascular system of individuals, but depends on the physiology of each person individually,
- "intense" exercise has been associated with an increased rate of sudden death, especially in individuals over 35 years old, with coronary artery disease being the main culprit, justifying the necessity of detecting health problems in chess players for the protection of their health through the issuance of Health Cards.

23.2. To satisfy the principle of proportionality, the Board of Directors of the KOA proceeded to



minimize the required examinations for the issuance of Health Cards by the athletes of the Cyprus Chess Federation (KYΣO), as shown in the table presented in the incidents of this Decision, also modifying the duration of validity. To justify the completion of the Medical Examination Form for the issuance of the Health Card, the ASYA Committee explained that the questions included, with the assistance of the electrocardiogram, aim to detect the existence of hereditary cardiovascular diseases in the athlete to protect them from the risk of a cardiovascular event at the time and place of exercise, thus achieving the purpose of issuing the Health Card.

24. Although the effect of mental sports on the cardiovascular system, particularly for chess, has not been studied in depth and the number of studies examining the effect of chess on the cardiovascular system is limited, as noted by the ASYA Committee, in my opinion, this does not justify the complete abolition of the issuance of the Health Card for this specific sport.

25. Respecting the medical aspect of the issue and the fact that the ASYA Committee justified its positions by providing specific explanations and scientific references, presenting the reasons why it deems the issuance of Health Cards for chess players necessary, and also modified the prerequisites for the issuance of Health Cards for the athletes of the KYΣO, I do not find any other changes to the medical data collected to be proportionate. I emphasize that since the issuance of Health Cards also involves issues of personal data, as the health data collected and evaluated constitute special categories of personal data whose processing is generally prohibited and is only legitimized in specific cases, the KOA, as the data controller, must monitor medical developments and update/modify accordingly its procedures related to the issuance of Health Cards, so that medical examinations and/or any medical checks for the issuance of Health Cards are determined based on the sport to which they pertain.

26. In reaching the above conclusion, I also take into account the presence of the Federations and their Cardiologists at the meetings concerning the determination of the required examinations when submitting the application for the issuance of Health Cards and I find it important that their views and recommendations on the matter are evaluated and taken into account by the ASYA Committee and consequently by the KOA. Regarding the claims of the Complainant, concerning the consideration of...  
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Other factors in the Federation's decision to comply beyond the usefulness of the specific checks for chess (a claim that was not further substantiated), I am of the opinion that if and when the Federation's positions are adequately substantiated, they should be heard.

27. In summary of the above, in examining the matter, on the one hand, specialists have been involved, different each time (both from the involved party (KOA) and from the federations) who assessed the necessity of examining the athletes; on the other hand, during the investigation, I requested and received the positions and opinions of the KOA as the data controller. Therefore, I did not find it necessary to obtain another opinion from an external expert.

28. As I have noted in my preliminary Decision, regarding the position of the ASYA Committee that -

- the process falls within the practices of medical practice,
  - the recording of pathological findings forms a database for research purposes for the benefit of reliable statistics and the formation of an incident registry, adhering to the rules of the bioethics committee,
  - the extraction of pathological data will contribute to the improvement of practices followed in the prevention of AKTH and the maximization of conditions ensuring the lives of citizens/athletes,
- I emphasize that although the above constitute a legitimate purpose of processing, compatible with the initial purpose that has been established (Article 5(1)(b) of the Regulation), the processing of data for scientific research purposes constitutes further processing of personal data. In any case, the personal data collected should be proportional to the purpose that has initially been established.

29. Also relevant is the following excerpt from the "EDPB Document on response to the request from the European Commission for clarifications on the consistent application of the GDPR, focusing on health research," which, although it concerns health care providers, I believe can be analogously

applied to the above position of the ASYA Committee:

“Q7. If a health care provider collected health data from patients and wishes to use those data for a scientific research project, to what extent would this be considered compatible further processing?

22. When further processing of health data for scientific research purposes relies on the presumption of compatible use (Article 5(1)(b) GDPR), the controller will have to take Article 9 GDPR into account as well. It could very well be that the exemption to the prohibition on the processing of health data the health care provider relied on for the original purpose does not extend to or does not cover the processing of health data for scientific research purposes. For instance, if the exemption in MS law only allows for the processing of health data by the health care provider in order to provide health care or medical treatments (Article 9(2)(h) GDPR), the health care provider would still need to rely on an exemption based on Union or MS law as required in Article 9(2) GDPR for the processing of health data for scientific research purposes.”

30. Regarding the safeguards that should be taken during the processing of data for scientific research purposes, the EDPS/2019/02-08 report titled: “Study on the appropriate safeguards under Article 89(1) GDPR for the processing of personal data for scientific research” is useful.

31. Regarding the processing of data related to the military service of athletes, which the Complainant raises in his latest letter, this constitutes a separate issue that is entirely based on the legal provision contained in the Cyprus Sports Organization Law of 1969 (41/1969). “It is understood that the health card is in no case issued if the athlete is conscripted who has been called up and enlisted in the National Guard, according to the provisions of the National Guard Law, from the year 2019 onwards and holds, under this Law, a temporary discharge, either due to unfitness for military service (I/5) or due to a granted postponement of fulfilling the obligation of military service, due to a disease/condition related to his mental health (...)”.

32. The above provision assigns specific responsibilities to the KOA. For this matter, I had previously provided relevant guidance to the Ministry of Defense and the KOA regarding the technical solution that would be implemented so that the National Guard would provide the KOA only with the necessary information required by Article 5(2) of Law 41/1969, the implementation of an impact assessment, ensuring that the National Guard would provide the KOA only with the data that is absolutely necessary for fulfilling its legal obligations and the inclusion of relevant text informing the data subjects. Therefore, the matter will not be reconsidered in the present complaint.

33. Finally, I consider that the Complainant's claim of coercion in data collection is directly examined by the re-evaluation of the proportionality of issuing Health Cards.

#### D. Conclusion

34. From the evidence before me, the above analysis, the modifications made by the Cyprus Sports Organization following the issuance of my preliminary Decision, and based on the powers granted to me by Article 58 of the

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In accordance with Regulation (EU) 2016/679, I conclude that the Cyprus Sports Organization has complied with the provisions of Regulation (EU) 2016/679, based on the findings of my preliminary Decision.

35. Taking into account future medical developments that may modify what has been reported by the Data Protection Authority, I recommend to the Cyprus Sports Organization to monitor this issue and take appropriate measures so that the processing activities it conducts comply with the Principles that must govern any processing of personal data, as defined in Article 5 of Regulation (EU) 2016/679. Should it be necessary in the future, I reserve the right to intervene and reassess the matter.

Peace Loizidou Nicolaidou

12 July 2024

Commissioner for Personal Data Protection